Prodigy Anesthesia Quick Review

Hepatic Surgery

Where does the majority of bleeding come from during a liver resection?	Intrahepatic branches of hepatic and portal veins that are injured during the dissection *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 564.*
Describe the Pringle maneuver.	The Pringle maneuver is the temporary occlusion of the hepaticoduodenal ligament. This ligament houses the hepatic artery, main portal vein, and the common bile duct. It is a technique used to minimize blood loss. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 564.*
What is the most important predictor of short-term survival post liver resection?	Intraoperative blood loss *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 564.*
Morbidity and mortality following a liver resection depends upon what factors?	The patient's liver function, the extent of the surgery, and the experience of the surgeon *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 564.*
What are the most common secondary tumors removed during a liver resection?	Metastases from colorectal cancer *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 565.*
What is the most common primary liver tumor? What leads to this type of tumor's development?	Hepatocellular carcinoma (HCC). Cirrhosis due to chronic alcohol abuse and chronic hepatitis B or C *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 564.*
What are the two main reasons liver resections are performed?	1. For the removal of primary liver tumors 2. For the removal of metastatic tumors to the liver *Dorsch JA, Dorsch SE. Understanding Anesthesia Equipment. 5th ed. Philadelphia, PA: Lippincott Williams and Wilkins, 2008: 550.*
List postoperative complications resulting from a liver resection.	Pulmonary insufficiency, DIC, hypoglycemia, hypothermia, hemorrhage, decreased liver function, electrolyte imbalances *Jaffe RA, Samuels SI. Anesthesiologist's Manual of Surgical Procedures. 4th ed. Philadelphia, PA: Lippincott Williams and Wilkins, 2009: 554.*

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Why would a patient with normal liver function prior to liver resection experience liver dysfunction postoperatively?	Because of ischemic injury resulting from vascular occlusion or from the loss of liver mass *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 569.*
Why is it prudent to know the size of the tumor and any involvement of vascular structures prior to a liver resection?	In order to adequately prepare for major intraoperative fluid and blood losses *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 568.*
Should coagulopathy be a concern in patients presenting for liver resection?	Yes. The liver manufactures all of the clotting factors with the exception of factor VIII. *Jaffe RA, Schmiesing CA, Golianu B. Anesthesiologist's Manual of Surgical Procedures. 5th ed. Philadelphia, PA: Wolters Kluwer Health, 2014: 568.*

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